PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	The effect of eHealth-delivered exercise programmes on balance in people aged 65 years and over living in the community: A systematic review & meta-analysis of randomised controlled trials.
AUTHORS	Ambrens, Meghan; Alley, Stephanie; Oliveira, Juliana; To, Quyen; Delbaere, Kim; Vandelanotte, Corneel; Tiedemann, Anne

VERSION 1 – REVIEW

REVIEWER	McGrath, Ryan North Dakota State University
REVIEW RETURNED	27-Jul-2021

GENERAL COMMENTS	BMJOPEN-2021-051377 presents results for a SR and MA on e-health exercise interventions for fall-related outcomes. While some parts of this manuscript were interesting, other areas could be improved. I hope the authors consider my feedback. • Abstract: Should be, “Eligible studies…” Correct typos throughout. • In text citation format should be “.1”? • Introduction: Recent findings from a detailed intervention aiming to prevent falls showed that persons in the fall prevention group did not result in a lower rate of falls that usual care (STRIDE study; https://www.nejm.org/doi/full/10.1056/NEJMoa2002183). Why would a less detailed intervention such as e-health better prevent balance concerns and falls? • Data Sources: The September 2020 search is arguably outdated and the search now needs updating. • Data Sources: This reviewer was not able to locate keywords used to find articles? • Outcome Measures: Balance and fall risk are uniquely different outcomes. It is probably in the best interest of the study to just remove these outcomes and focus on balance. • Table 1: Most participants in the studies included were healthy. How might unhealthy older populations benefit from an e-Health exercise intervention if at all? • Table 1: Is it possible that the Intervention contents and FITT characteristics factored into balance more so that the platform of delivery? This may have more importance than mode of intervention dissemination? • Discussion: Should we be providing resources and training to older adults to engage in an e-Health platform? How might other health characteristics benefit from e-Health? Does the COVID-19 pandemic further support e-Health?

REVIEWER	Arnold, Susanne University of Warwick

REVIEW RETURNED	12-Oct-2021

GENERAL COMMENTS	This is a very important and relevant systematic review to understand the impact of eHealth delivered exercise on balance in older people. Abstract: Page 4, Line 15: typo - trials not trial Page 4, Line 22: you have missed out a few words describing when the searches were undertaken. Introduction: clear rationale for the review. Methods: Outcome measures: I appreciate that you have included more detail about the outcome measures in your protocol but I think a brief description of what you mean by static and dynamic balance and some examples of the measures that might be used to measure these would be useful to give the reader some context. Results: Page 13, Line 35: You have used the abbreviation SMT but have not given a full description of what SMT stands for? Did you consider doing any other analysis i.e. studies using the same eHealth intervention such as the Wii or combining results of studies using one specific outcome measure. I appreciate these would have been very small analyses - just two or three studies with possible very small number so participants - but I just wonder if you had considered this? It may have been interesting to see if one particular intervention was driving any effect but again the numbers may have been to small to determine any definite results? You have stated in your methods that also extracted data on drop out, implementation fidelity, and intervention acceptability but you haven't commented on those in the results. Discussion: I think that your discussion about dose and intensity of the prescribed exercises is very pertinent and an ongoing issue with a lot of exercise interventions. You have touched on the fact that the sample sizes in some of these trials may not have been powered sufficiently to detect a difference on falls, but might this also have been true of balance outcomes. I think you might want to comment on the very small sample sizes in a number of these studies as another potential limitation. You have said that clinicians should consider the use of eHealth platforms for delivering exercise programmes for older people, but could you be more specific i.e. would you recommend the Wii over the X Box or are you saying that any of them will have the same effect (this is where further analyses may have been helpful). I appreciate you may not wish to endorse one particular brand but am just thinking about this from the point of view of a clinician who thinks they might want to use eHealth but then which brand/type should they use as there is still some heterogeneity between interventions?

REVIEWER	Haas, Mitchell University of Minnesota

REVIEW RETURNED	02-Dec-2021

GENERAL COMMENTS	This reviewer focuses on the analysis, its interpretation, and its presentation. A minimum of clarification is requested. Abstract 1. Results: a. The magnitude of the SMD is of primary importance, so include descriptors (i.e., small, medium, large). b. It is not strictly correct to say “no effect” in the absence of statistical significance. An SMD of 0.2 is considered a small effect. 2. Conclusion: a. Include both primary outcomes in the conclusion. b. Include the strength/quality of evidence and descriptors of magnitude of the effect size. c. It would be helpful to call the evidence “preliminary” here, as is most appropriately expressed in the Discussion. Methods 3. Statistical Analysis: a. Briefly explain how I-squared was used in the determination of statistical heterogeneity. For example, I-squared > 50%. b. State why the random-effects model was chosen over the fixed-effects model (e.g., statistical or clinical heterogeneity). Discussion 4. Feasibility and Safety: The intervention is identified as feasible and safe twice in the Discussion. The assertion needs to be reframed because the language can easily be mistaken to mean that feasibility and safety were fully evaluated using meta-analysis. Feasibility can, to some extent, be implied by the success of the intervention evaluated by meta-analysis, but feasibility usually refers to adherence which was not evaluated. Establishment of safety is more of a concern in that analysis of adverse events would be required in a much larger sample. 5. Implications for clinicians and policymakers: This section needs to be put in the perspective of the quality of evidence. Policymakers in particular should be aware that the GRADE for the two primary outcomes was low and very low.

REVIEWER	Morelhão, Priscila K Universidade Federal de São Paulo, Psicobiologia
REVIEW RETURNED	10-Dec-2021

GENERAL COMMENTS	The authors submitted the article entitled “The effect of eHealth-delivered exercise programmes on balance in people aged 65 years and over living in the community: A systematic review & meta-analysis of randomised controlled trials.” This study aimed to synthesize the evidence and evaluated the effect of eHealth-delivered exercise programmes on balance in people aged 65 years and older living in the community compared to a control. Nine articles were included in meta-analysis and the results showed that eHealth-delivered exercise programmes had an effect static balance, but there was not effect on dynamic balance. This article was very well conducted, and the methodological rigor was high. I have minor questions about this article. 1-Why did the authors include people aged 65 and over and not aged 60 or over? What is the definition by age that the authors consider the older person?

	2-Did the authors include quasi-randomized studies? This needs to be included in the methods. 3-Do the authors believe that based on the results of this work, the eHealth platform could be a treatment strategy in telerehabilitation? 4-Could the fact that some studies have not supervised the older adults could have been a problem for the dynamic balance?

VERSION 1 – AUTHOR RESPONSE

Editor's comments
Please include an example of the full search strategy for one database as a supplementary file.
Response: The search strategy for MEDLINE has been included as Supplementary file 1

ABSTRACT:
Should be, "Eligible studies…" Correct typos throughout.
Response: This has been corrected throughout the manuscript.

In text citation format should be ".1"?
Response: This has been corrected throughout the manuscript.

Page 4, Line 15: typo - trials not trial
Response: This has been amended throughout the manuscript with the request to replace the word trial with study.

Page 4, Line 22: you have missed out a few words describing when the searches were undertaken.
Response: The manuscript has been edited to say:
"Nine databases including MEDLINE, CINAHL, and Embase, were searched from inception to January 2022 to identify randomised controlled trials evaluating eHealth-delivered exercise programmes for community-dwelling people aged ≥65 years, published in English that included a balance outcome."

Results:
a. The magnitude of the SMD is of primary importance, so include descriptors (i.e., small, medium, large).
Response: The manuscript has been edited in the results section to add the descriptor for magnitude of the SMD for each outcome.

b. It is not strictly correct to say "no effect" in the absence of statistical significance. An SMD of 0.2 is considered a small effect.
Response: This has been changed to add the descriptor for the SMD for each outcome reported in the manuscript.

Conclusion:
a. Include both primary outcomes in the conclusion.
Response: The manuscript has been edited as follows:
"This review provides preliminary evidence that eHealth-delivered exercise programmes improved balance and reduced fall risk in people aged ≥65 years. There is still uncertainty regarding the effect of eHealth delivered exercise programmes on fear of falling."

b. Include the strength/quality of evidence and descriptors of magnitude of the effect size.

Response: We have included the strength/quality in reporting the results.

c. It would be helpful to call the evidence “preliminary” here, as is most appropriately expressed in the Discussion.
Response: The manuscript has been edited as follows:
“This review provides preliminary evidence that eHealth-delivered exercise programmes improved balance in people aged ≥65 years.”

INTRODUCTION:
Recent findings from a detailed intervention aiming to prevent falls showed that persons in the fall prevention group did not result in a lower rate of falls that usual care (STRIDE study; https://www.nejm.org/doi/full/10.1056/NEJMoa2002183). Why would a less detailed intervention such as e-health better prevent balance concerns and falls?
Response: The intervention in the STRIDE study is a multifactorial intervention consisting of a fall risk assessment and implementation of an individualised treatment/intervention plan to address risk factors identified. It did not provide an exercise-based programme for participants. There are many factors that influence a person’s risk of falling. A multifactorial intervention provides more than one treatment/intervention to address a person’s risk factors for falling. Our systematic review seeks to evaluate the effectiveness of a technology-delivered exercise-based intervention on balance (a single factor intervention for improving balance and reducing falls risk). The 2019 and 2018 Cochrane reviews found that exercise was the single most effective intervention for improving balance to reduce falls in older people (Sherrington et al., 2019; Hopewell et al., 2018). These reviews also confirmed it is the relationship between exercise dose and frequency that is integral to effectiveness, rather than level of intervention detail. This systematic review also aimed to evaluate the effectiveness of a technology-based platform to deliver an exercise-based intervention.

METHODS:
Did the authors include quasi-randomized studies? This needs to be included in the methods.
Response: We have not included quasi-randomised studies in this review as quasi-randomised studies use a method of allocation that is not truly random. Studies included in this systematic review met the following criteria: (1) published in English, (2) randomised controlled trials (RCT), (3) participants were community-dwelling people aged ≥65 years, (4) reported data for a validated measure of balance, (5) included eHealth delivery of an exercise programme compared with no intervention, usual care or wait-list control. Studies that did not meet these criteria were excluded.

We included all RCT designs such as crossover, cluster, patient-randomised clinical trials that examined the effect of eHealth-delivered exercise programmes versus a control group. Single and multi-factorial interventions were also included. Studies published only as abstracts or yet to be published were excluded due to possible data inaccuracy and incompleteness.

Why did the authors include people aged 65 and over and not aged 60 or over? What is the definition by age that the authors consider the older person?
Response: The definition of ‘older age’ varies considerably in the literature. We selected age 65 and above since this aligns with the retirement age in many countries and because statistics on falls often align with this age.

Data Sources:
The September 2020 search is arguably outdated and the search now needs updating.
Response: We have updated the search to include publications published from inception to January 2022.

This reviewer was not able to locate keywords used to find articles?
Response: The manuscript has now been edited to include the key word search strategy as Supplementary file 1.

Outcome Measures:
Balance and fall risk are uniquely different outcomes. It is probably in the best interest of the study to just remove these outcomes and focus on balance.
Response: We acknowledge the reviewer's comment. However, we have decided to keep the secondary outcomes of fall risk and fear of falling in this review. Balance is used as a proxy measure of the possible impact of an intervention on falls in circumstances where resources do not allow for the conduct of an adequately powered trial, (which would require approximately 500 participants), with falls as the primary outcome. Research has found that balance is an important functional measure and strong predictor of falls risk (Tiedemann et al., 2008).

I appreciate that you have included more detail about the outcome measures in your protocol, but I think a brief description of what you mean by static and dynamic balance and some examples of the measures that might be used to measure these would be useful to give the reader some context.
Response: The manuscript has been edited to add a definition for balance, static balance and dynamic balance:
The primary outcome measure was balance, defined as staying upright and steady when stationary, such as when standing or sitting, or during movement (Howe et al., 2011). Technically, balance is defined as "the ability to maintain the projection of the body's centre of mass (CoM) within manageable limits of the base of support, as in sitting or standing, or in transit to a new base of support, as in walking" (Winter, 1995).

Static balance refers to maintaining balance when the body has a constant or static base of support, such as when standing with feet fixed in one position (Howe et al., 2011). Whereas dynamic balance refers to maintaining balance during movement from one base of support to another, such as when walking (Howe et al., 2011)

Table 1:
Most participants in the studies included were healthy. How might unhealthy older populations benefit from an e-Health exercise intervention if at all?
Response: Our results apply to general samples of older people, many of whom would have underlying comorbidities. It is outside the scope of this review to comment on the specific effects (benefits and harms) of e-Health exercise interventions for clinical populations. Further research is needed to determine the specific benefits of e-Health exercise interventions when tailored to the needs of people with specific clinical diagnoses.

Is it possible that the Intervention contents and FITT characteristics factored into balance more so that the platform of delivery? This may have more importance than mode of intervention dissemination?
Response: Frequency, Intensity, Time, and Type are important principles of all exercise programmes. Specific to a fall prevention programme, Ng et al. (2019) found that different exercise programmes may have different effects on falls in people aged 65+ years, therefore careful analysis of programme characteristics and their impact is important. Sherrington et al. (2016) found that exercise programmes that challenge balance (i.e., by reducing base of support, moving the centre of gravity, standing without arm support) and are of a higher dose (i.e., ≥3 hours each week) have larger effects than others. The Cochrane reviews by Sherrington et al (2019) and Hopewell et al (2018) confirmed this, finding that it is the relationship between exercise dose and frequency that is integral to effectiveness. Despite the confirmation that exercise type, dose and frequency is important it is not the only consideration. What is also of importance is what is delivered and how it is delivered. As stated in our introduction, long-term adherence to an exercise programme designed to improve

balance and reduce falls risk is poor. While effectiveness has been proven in both the home-based and group-based settings research has identified barriers to both forms of delivery, which may be influencing adherence. This suggests we need to provide people with options, as not all delivery methods are acceptable and effective for all people. Therefore, this study has focussed on the important need to explore the effectiveness of a novel delivery method that has a wide reach and may address some of the barriers to adherence, especially considering the COVID-19 pandemic.

Statistical Analysis
a. Briefly explain how I-squared was used in the determination of statistical heterogeneity. For example, I-squared > 50%.
Response: Table 2 provides a summary of the quality of evidence and strength of recommendation. It outlines that heterogeneity as measured by I2 is >50%.

b. State why the random-effects model was chosen over the fixed-effects model (e.g., statistical or clinical heterogeneity).
Response: We chose the random-effects model because it is reasonable to assume that true effect will vary from study to study, given the heterogeneity of the population and the interventions we are evaluating. The manuscript has been updated to reflect this, stating:
“The random effects model was chosen given the heterogeneity of the population and the interventions being evaluated.”

RESULTS
Page 13, Line 35: You have used the abbreviation SMT but have not given a full description of what SMT stands for?
Response: The manuscript has been edited to include a description for SMT as follows:
Step-mat-training (SMT)

Did you consider doing any other analysis i.e., studies using the same eHealth intervention such as the Wii or combining results of studies using one specific outcome measure. I appreciate these would have been very small analyses - just two or three studies with possible very small number so participants - but I just wonder if you had considered this? It may have been interesting to see if one particular intervention was driving any effect but again the numbers may have been to small to determine any definite results?
Response: We agree that this would have provided interesting insights into the relative effects of different intervention approaches, however as you indicated, there were too few studies to allow this approach.

You have stated in your methods that also extracted data on drop out, implementation fidelity, and intervention acceptability but you haven't commented on those in the results.
Response: We agree that this would have provided interesting insights into the acceptability of the method for delivering an exercise program there were too few studies to allow for any meaningful comment. Only three studies (out of the 14) reported they assessed intervention acceptability. We have added a column to Table 1 outlining if intervention acceptability was assessed and if so, how it was assessed.

DISCUSSION:
Should we be providing resources and training to older adults to engage in an e-Health platform? How might other health characteristics benefit from e-Health? Does the COVID-19 pandemic further support e-Health?
Response: We have found that older people are interested in and have the capacity for learning how to successfully engage with technology (Ambrens et al., 2021; Delbaere et al., 2021). Nevertheless, eHealth interventions need to be designed so that they can be comfortably used with a minimum of

technical knowledge and ongoing support. Further research is needed to determine how to best support older people to confidently use technology, to support their health, education and wellbeing needs (Ambrens et al., 2021).

It is outside of the scope of this review to comment on how other health characteristics may benefit from e-Health. However, the COVID-19 pandemic has demonstrated that increased social distancing and stay at home orders led to a decrease in physical activity levels and the increased likelihood of a negative spiral of low functional capacity and higher risk of falls and other adverse health events in this population (McGarrigle & Todd, 2020). Telehealth and other technology-based platforms have an integral role in allowing for the continued delivery of health and medical services.

I think that your discussion about dose and intensity of the prescribed exercises is very pertinent and an ongoing issue with a lot of exercise interventions. You have touched on the fact that the sample sizes in some of these trials may not have been powered sufficiently to detect a difference on falls, but might this also have been true of balance outcomes.
I think you might want to comment on the very small sample sizes in a number of these studies as another potential limitation.
Response: The manuscript has been edited as follows:

You have said that clinicians should consider the use of eHealth platforms for delivering exercise programmes for older people, but could you be more specific i.e., would you recommend the Wii over the X Box or are you saying that any of them will have the same effect (this is where further analyses may have been helpful). I appreciate you may not wish to endorse one particular brand but am just thinking about this from the point of view of a clinician who thinks they might want to use eHealth but then which brand/type should they use as there is still some heterogeneity between interventions?
Response: Since a comparison of effectiveness between different types of e-Health interventions was not possible in this research, we are unable to recommend one type over another. Our overall recommendation is really alluding to the fact that these approaches appear to be well received and effective for some older people and that clinicians should consider them if they are available and of interest to their older patients.

Do the authors believe that based on the results of this work, the eHealth platform could be a treatment strategy in telerehabilitation?
Response: Yes. The COVID-19 pandemic has demonstrated that the reduced access to health care is a serious public health concern. McGarrigle and Todd (2020) found the increased social distancing and stay at home orders resulting from the COVID-19 pandemic led to a decrease in physical activity levels and this increased the likelihood of a negative spiral of low functional capacity and higher risk of falls and other adverse health events in older people. Furthermore, a review of the impact COVID-19 has had on access to health services (unrelated to falls prevention in older people) has found that the delayed access to hospital care due to patients fears of infection resulted in admission to an intensive care unit and even death (Lazzerini et al., 2020). In addition to recent global events, there are numerous other factors influencing an individual's ability to access health services, including but not limited to poverty and geographical location (Child et al., 2012; Davies et al., 2012). Therefore, the confirmation that technology-delivered platforms such as eHealth are an effective method for delivering health and rehabilitation services is important to improving access to health services and in turn health outcomes of individuals.

Could the fact that some studies have not supervised the older adults could have been a problem for the dynamic balance?
Response: The update to the meta-analysis requested by the reviewers has changed the results related to dynamic balance. The pooled effect of eHealth-delivered exercise programmes on dynamic balance indicates there was a small, statistically significant effect compared to control (SMD = 0.42,

95% CI 0.11 to 0.73; I2 = 79%, p = 0.009). There are a number of factors which may have influenced this result and more research is needed to understand these factors. What we do know is that the Cochrane review found exercise programs delivered in a supervised group setting as well as those delivered individually with minimal supervision were equally effective in preventing falls (Sherrington et al., 2019). Thomas et al (2020) also argue that it is important to support health professionals to gain skills and confidence in using technology as a treatment platform, which is a key requirement for the successful long-term sustainability of technology-driven platforms.

Feasibility and Safety: The intervention is identified as feasible and safe twice in the Discussion. The assertion needs to be reframed because the language can easily be mistaken to mean that feasibility and safety were fully evaluated using meta-analysis. Feasibility can, to some extent, be implied by the success of the intervention evaluated by meta-analysis, but feasibility usually refers to adherence which was not evaluated. Establishment of safety is more of a concern in that analysis of adverse events would be required in a much larger sample.
Response: The reference to feasibility and safety was removed and the manuscript has been edited in the following places:
Discussion:
“This systematic review and meta-analysis provides preliminary evidence that eHealth-delivered exercise programmes improve static balance and are an alternative method of delivering an exercise programme to people aged ≥65 years living in the community.”

Implications for clinicians and policymakers:
“This review demonstrates that eHealth platforms are an effective mode of delivering exercises to improve balance.”

Implications for clinicians and policymakers
This section needs to be put in the perspective of the quality of evidence. Policymakers in particular should be aware that the GRADE for the two primary outcomes was low and very low.
Response: This section of the manuscript has been updated to include a comment about the quality of evidence:
Although we identified an intervention effect on balance (static and dynamic), not only does this need to be considered in the context of the low quality of the evidence but also in the ability to scale-up and implement such interventions to large populations where resources are available.

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VERSION 2 – REVIEW

REVIEWER	Arnold, Susanne University of Warwick
REVIEW RETURNED	20-Apr-2022

GENERAL COMMENTS	Thank you for answering all of my comments and queries satisfactorily.

REVIEWER	Haas, Mitchell University of Minnesota
REVIEW RETURNED	19-Apr-2022

GENERAL COMMENTS	The authors have addressed all of this reviewer's comments except for one word inadvertently missing. Discussion Implications for clinicians and policymakers:

	In the response to reviewers, the authors state that changes were made so that the text would read as follows: “Although we identified an intervention effect on balance (static and dynamic), not only does this need to be considered in the context of the low quality of the evidence but also in the ability to scale-up and implement such interventions to large populations where resources are available.” In the manuscript, the word “low” is missing.

REVIEWER	Morelhão, Priscila K Universidade Federal de São Paulo, Psicobiologia
REVIEW RETURNED	02-May-2022

GENERAL COMMENTS	Manuscript accepted, all necessary changes were incorporated.